

Institute of Actuaries of India

Subject SA1 – Health and Care Insurance

May 2012 Examination

INDICATIVE SOLUTION

Introduction

The indicative solution has been written by the Examiners with the aim of helping candidates. The solutions given are only indicative. It is realized that there could be other points as valid answers and examiner have given credit for any alternative approach or interpretation which they consider to be reasonable.

Solution 1 :**A)**

Customer Lifetime Value: Customer lifetime value is a concept used across many industries including insurance and other segments in financial service sector. Calculation of Customer Lifetime Value (CLV) is an attempt to quantify the profit that a customer is expected to bring to the insurer, taking into account the entire future lifetime of his needs and the extent of cross-selling possible.

Uses of Customer Lifetime Value: The concept of CLV can be put to the various uses, some of which are:

- 1) Segmentation of policyholders on the basis of CLV helps insurer in better planning with respect to the level of services on offer. In case of tiered service, the most valued customer segment can be provided the most expensive services and so on.
- 2) It helps planning in better intermediation level. For example, higher commission and brokerage (initial and renewal) can be offered in case of customers who have higher CLV even if the initial premium paid by them may not be very high.
- 3) It helps in nurturing the profitable customer segments.
- 4) It shifts focus from immediate and short term to long term perspective.
- 5) It helps in making best use of marketing budget by directing the advertising campaigns, etc. towards the most profitable segment in terms of CLV.
- 6) The movement of total CLV and that of different customer segment may provide good insight as far as direction of state of affairs of the company is concerned.
- 7) It helps in monitoring the portfolio in a better way. The whole definition of profitability changes and the company may take decision based on the CLV instead of other individual parameters. As a matter of fact, CLV very well incorporates many other parameters, like, loss ratios, renewal rate, cost of capital, etc.
- 8) It may help in company's valuation although the sole purpose is not that.

B)

Computing Customer Lifetime Value: CLV calculation involves computation of all cash flows items that are going to take place during lifetime of the customer, like, premium receipt, claim payment, payment of commission, various expenses, etc. The renewal propensity of the customer segment plays an important role here. The timings of cash flow need to be ascertained and the future cash flows are discounted by risk discount rate.

The computation of CLV can be studied under the following headings:

1) Identify items of cash flow: The first step is to ascertain the inflows and outflows involved in any policy. Some of the major items of cash flows are given below:

- Premium Income
- Commission/ Brokerage
- Underwriting and policy set-up cost
- Claims payment
- Claims handling expenses
- Reinsurance cost
- Expenses to service policy
- Apportioned office overheads
- Allocated management expenses, etc.
- Taxes

Above list is certainly not exhaustive but covers major heads of cash flows involved in a health insurance policy.

2) Customer Segmentation: Second step is classification of customers in different segments. The very purpose of computation of CLV should determine the parameters used in segmentation of policyholders. A few of the parameter could be sum-insured, age-band, gender, occupation, annual family income, geography, etc. The segmentation could be one-way, two-ways or more refined. Care needs to be taken that segments are not too many to make the assessment of parameter value for these segments less credible, thus, computation of CLV for each segment less reliable.

3) Determining Parameter Values: Next step is to determine parameter value for the items of cash flows for each segment. Example of parameters whose value needs to be ascertained are claim incidence rate, average claim size, rate of renewal of policy, commission rate, capital required, investment income, administration expenses, inflation (claim and expense), etc. It is important to note that not only the value of these factors needs to be determined in present but these are required to be forecasted in future as well. For few of these factors like, incidence rate, average claim size, renewal rate, etc. we can look at actual experience of the portfolio for different segments. For items like, commission (new and renewal) we can use present rate in force. For predicting investment income, inflation, etc. in future it is better to be slightly prudent. Existing

solvency norms may give idea about capital required. For allocation of management expenses on policies pricing assumption may be helpful. In some cases it may be difficult to identify parameter values for different segments of the portfolio directly and better approach will be to use multivariate technique to take care of correlations between different factors. Revision in premium rates in future also needs to be determined. One simplistic assumption could be to assume that deterioration in performance of the portfolio because of upward trend in utilization and inflation will be offset by future increases in premiums. This assumption may not be unreasonable if the premiums are fully reviewable.

- 4) Deciding Risk Discount Rate:** Risk discount rate plays an important role in calculation of CLV. It is the rate which is used to discount the future cash flows at present level. There are many factors that influence the determination of risk discount rate, like, general level of interest rate prevalent in the economy, cost of capital, risk free rate, level of riskiness of the portfolio, shareholders' expected return, etc. For a segment of portfolio where it is difficult to review rates due to various reasons, a higher risk discount rate may be used compared to one where premium review is quite easy.
- 5) Computation of CLV:** Once we have determined the parameter values for each segment and zeroed in on Risk Discount Rate the calculation of CLV can be done easily. Future cash flows can be predicted for each segment based on various factors identified in the steps above. Renewal rate plays an important role here which tells us how this portfolio of annually renewable policies is going to run off in future. If the no. of segments is small and cash flows are also not many it can be done easily using a spreadsheet. But more precise calculation at more granular level may warrant use of software designed for this.

Up-sell and Cross-sell of different products to customer can also be considered in the CLV model. This can be easily incorporated in the model if we have historical experience of up-selling and cross-selling. The probability of policyholder buying higher value products and more than one product can be estimated based on historical data for different segments. Further calculation is quite easy as we already have cash flows for different products variants in place in our model.

- 6) Reasonability Checks:** Once the CLV is calculated for different segment of the portfolio, it is important to check them whether these are within reasonable limits or not. Results should be compared with the other monitoring reports of the company consisting of loss ratio, etc. If any inconsistency is noticed the same needs to be looked into and resolved accordingly.

CLV needs to be analysed further. If CLV is coming hugely negative for a particular segment of the portfolio which is sizeable, the action plan is required to deal with it.

C)

Embedded Value (EV) & Customer Lifetime Value (CLV): EV and CLV are both similar in approach, since in the core of both the concepts is discounting of future cash flows to the present value using appropriate Risk Discount Rate.

However, these two concepts vary in quite a few respects as given below:

1. EV is defined as the value of the future profit stream from the company's existing business plus the value of any net assets separately attributable to shareholders. Hence, it is used more from the shareholder's point of view and considers company as a whole. It also considers total assets in the balance sheet net of liabilities. CLV may be calculated for different segments in the portfolio of some particular product and does not consider other assets and liabilities of the company.
2. The parameter values (risk discount rate, inflation, investment income, etc.) used in EV and CLV may vary a lot. In case of EV it may depend on the purpose of reporting and may also be governed by legislation in force if any while for CLV it may be on the best estimate basis or may use different scenarios. In case of CLV the idea is to bring out difference between different customer segments or to monitor it over a period of time and, hence, parameter values are not that important (as in EV) as long as these are reasonable and used consistently.
3. EV can be used for external reporting purpose while CLV is more an internal concept to be used within the company to assist decision making.
4. CLV may use cross selling and up selling prospects which may not be used in EV.
5. EV is more of a finance concept while CLV is more of a marketing concept.

CLV may be calculated even for prospective customers while in EV only existing policies contribute.

[17]

Solution 2 :**(A)**

Badging: Badging is a concept where a large insurance company with significant marketing presence enters into an arrangement with a specialist health and care insurer, so that, the insurer provides the policy to the customer but the specialist insurer issues the policy and undertakes all policy processing and claims administration, payments, etc. Badging is a way of reducing insurance risk and thus reducing the need for additional capital. Many a times, badging is described as an alternative to reinsurance. However, it is not necessarily an alternative to reinsurance in the strict sense of the word, since it is simply a way of getting access to someone else's expertise without taking on any insurance risk yourself. It is different from reinsurance in that, for example, all of the risk is taken on by the specialist insurer, whereas under reinsurance the issuing company would take on the risk, which may be passed on to the reinsurer. In most of the regulatory frameworks, it is mandatory to mention names of both the insurance companies in marketing literature, so that, insured can take better informed decision.

B)**Advantages:**

1. It may be win-win for both. General Insurer in this case may be benefited with the expertise of Specialist Health and Care Insurer. At the same time, Specialist Health Insurer may be benefited with the marketing strength of the large General Insurance Company.
2. Health is a very complex line of insurance and it may take time and resources for General Insurer to build expertise in underwriting, claims management, etc. if it decides to go solo.
3. General Insurer can focus just on distribution which maybe its core competency.
4. It may be very effective approach in terms of 'time to market'. The product may be developed and marketed fairly quickly with the pooling of resources of the two companies.
5. Specialised insurer already has lot of capabilities in various functions and thus may reap benefit of economy of scale with the additional business under the new arrangement. This may result in lower management cost and will be in benefit of both.
6. General Insurer has large base of customers and thus can cross-sell health policies to existing policyholders.

Disadvantages:

1. It may not be profitable in the long run if insurer wants to get into Health and Care line of business. In that case, investing in building own expertise will be a better approach.
2. The critical processes like, underwriting, claims administration, operations, etc. rest with specialist health insurer and, so, General Insurer runs a reputational risk in case specialist insurer does not do a good job with the policyholders. This may impact brand of the insurer, more so, if it is cross selling to its existing customers.
3. There may not be direct link between sales force who are on the front and people working in operations and underwriting functions who are on the back end.
4. Specialised Health Insurer may face situation of conflict of interest with its own products and may take decisions that may not be in the best interest of other insurer if proper control is not exercised.
5. Any loopholes left in contracts with the specialist insurer may land the company in trouble.
6. There may be issues once the badging contract is over regarding who 'owns the customers'.

c)

Alternatively, General Insurer may decide to build in house capabilities in functions like, underwriting, claims, policy administration, etc. related to health insurance. This may be time consuming but may be thought of like building intellectual capital which will be beneficial in the long run. In terms of sharing of risk, go for reinsurer arrangements. If it decides to go for reinsurance, it may take help of reinsurer in other technical aspects also, like product development, product pricing, medical underwriting guidelines, etc. In order to meet the capital requirements it may get into arrangement of Financial Reinsurance.

If it has enough resources, it may also explore the possibilities of taking over a small sized insurance company with expertise in health and care insurance.

Alternative approach (building in house or take over) only makes sense if the General Insurance Company wants to be in the Health Insurance line in the long term. It also depends on the importance it places on health insurance compared to other lines of insurance it is presently operating in. If there is long term commitment from shareholders and top management to build sizeable health insurance portfolio in near future, it may be economical and strategically more effective to build in house expertise rather than going for badging arrangement with the specialised health insurer.

Solution 3 :**A)**

Product Development Considerations: Following are considerations for developing such product-

1. The first consideration is which all alternative systems of medicine should be included in the product. It should be clearly mentioned in the policy wording and other sales literature to avoid any mis-selling or dispute at a later stage.
2. It should be clarified whether insured can avail these benefits as inpatient or outpatient also. This will have significant impact on the pricing of this benefit.
3. In case of cashless claims, do we have providers of alternative system of medicines in our network or not. What is the density of such providers and is it concentrated in a particular geography?
4. Whether benefit is to be offered to both the existing and new policyholders or to new ones only. If it is to be offered to existing policyholders mandatorily whether we have flexibility to charge higher premium because of this extra benefit in view of regulatory issues and risk of increase in non-renewal.
5. If this benefit is offered on optional basis (in form of rider or separate variant), is there any chance of adverse selection.
6. Finally, a more objective study is required in the form of customer survey, etc. to make sure that there is really demand for such benefit before embarking on the product development.

Pricing Considerations: Following are considerations in pricing such product-

1. We have to be cognizant of the fact that if insured is taking treatment from alternative system of medicines his claims under usual (allopathic) would go down. Hence, we need to only consider difference between the two or if we are adding risk premium for new benefit we need to deduct the saving in our usual claims.
2. If there is some credible data of incidence rate and average claim size of these benefits, especially, of insured population the same can be used in assessing the risk premium for this benefit.
3. If insured population data is not available at all, we can look for national statistics and use the same with appropriate adjustment.
4. If we have ever extended this benefit in any other product, like Health coverage to corporate customers, the data from same can be used with proper adjustment.

5. We can also look for industry statistics for pricing this benefit or take help of re-insurer if they have some experience of such a product. Overseas data or data from consultants can also be of some help.
6. It is appropriate to build contingency margin in pricing for this product due to uncertainty because of incomplete information.
7. If there are similar products available in the market, a comparison can be made with suitable adjustment.

B)

Safeguards: The following are safeguards in this respect-

1. **Sub-limit:** Initially it is prudent to put some sub-limit for this benefit. Once we have some experience, we can think of enhancing this limit or covering the benefit to the extent of full sum insured.
2. **Co-pay:** Co-pay not only acts as a measure to reduce claim outgo but also alters the behavioural aspects of the insured thereby resulting in more than proportional saving.
3. **Deductible:** Deductible results in saving in claims outgo as well as help in avoiding small claims and hence, results in saving in claims and claim handling expenses. Moral hazard is also reduced with deductible in place.
4. **Monitoring:** The importance of monitoring of emerging experience cannot be over emphasised in this situation. As we are pricing this benefit with incomplete information, we need to monitor the performance of this benefit closely. For this we need to have proper IT system to capture policy and claims data in the way that it can be analysed later on.
5. **Reviewability:** Such benefit is to be offered in the products which are reviewable in terms of both premium and benefit structure.
6. **Pilot Launch:** It is in the interest of risk control to launch the product with this new benefit in a phased manner. Initially, this benefit may be offered in a limited geography or with some specific product only. The learning from pilot launch may be used in launch in other areas and, thus, company can avoid exposing itself to larger risk.
7. **Reinsurance:** Take appropriate reinsurance to spread the risk of unfavourable claim experience.

- 8. Pre-authorization:** If the base product does not have pre-authorisation as a mandatory clause, it may be advisable to make pre-authorisation compulsory for this benefit.

[10]

Solution 4 :

It seems that the person concerned has attributed the entire increase in average claim size in last year to inflation and further recommended the increase in premium equivalent to that. There seems to be some flaws in this line of thinking and further investigation and analysis are required as given below:

Checking the accuracy of the values: The first step is to check whether the calculation of the average claim size is correct or not in both the year. Apart from mathematical accuracy in terms of formula, etc. it is also important to check if the two figures compared are consistent or not. For example, it may happen that while in previous year most of the claims have been reported and so accounted for; in the current year some claims may be shown in outstanding. While comparing the average claims size either only paid claims of both the years should be taken into consideration or if outstanding claims of current year are also considered these are suitably adjusted if required (for example for co-pay, non-payable items, etc.).

Slicing and dicing data: Overall average claim size for entire portfolio may not be true representative of inflation. We need to divide data in small homogeneous groups. Classification can be by sum-insured, age-bands, geography, channel, etc. Balance should be maintained between preciseness and credibility of data while dividing into groups. This will give good insight about the movement in average claim size. For example, if average claim size is increasing in almost all the groups by approximately same rate, it may be actual increase. On the other hand, if the average claim size in individual classes has not increased but the overall average claim size has gone up because we are getting more claims from that part of portfolio where average claim size is higher (older policyholders, higher sum insured, metropolitan cities, etc.) the impact is not because of inflation but movement in portfolio. In this case, movement in exposure should be analysed. It is possible that we have started selling policies to those people where average claim is higher.

Changes in Disease Pattern: The total claims received by count and amount should be analysed by ICD codes in both the years. It should be noted whether disease pattern has changed of claimants in 2011 as compared to 2010. If this is the case, the reasons should be further investigated and a comparison should also be made with that of industry or competitor if reliable data is available. Similarly, average claim size should be compared by ICD codes and if there is an increase in a particular surgical procedure/ treatment of specific disease, the reasons for same should be analysed. It is also important to find out whether such increase is going to stay in future also or not.

One off events: The outbreak of some disease in one year may increase or decrease average claim size depending on whether treatment of such disease cost more or less than average claim size. It is better to remove such impacts in taking re-pricing decisions which are longer term in nature.

Average Claim Size is not a sole criteria: For premium revision, we need to analyse many parameters and average claim size is one of them. Other important factors to be looked into are loss ratio, incidence rate, claim per capita, etc. As explained in the point above the shift in portfolio may increase average claim size but loss ratio may not go up if we are charging appropriate premiums to those policyholders. Moreover, these actual parameter values should be compared with that of expected ones to come to any conclusion. For premium revision the loss ratio should be projected for future period considering various factors including but not restricted to inflation and compared with our acceptable loss ratio considering expenses and expected profit. The decision of change in premium should be taken based on comparison of projected loss ratio and acceptable loss ratio not merely on trend of average claim size.

Increase may not be uniform: Even if the decision is taken to increase the rates by close to 20% considering impact of various factors explained above. The increase in premium may not be same for all the plans. For this purpose a detailed re-pricing exercise need to be done to compare burning cost of these plans with the existing premiums. Adjustment may be made considering other factors like, acceptability by existing policyholders, competitors' rates, regulatory constraints, underwriting cycle, etc.

Provider Contracting: Management of provider network also plays an important role in controlling average claim size. It is to be further investigated whether average claim size has gone up for network hospital or non-network hospital or both and what is the extent of increase in each case. If there is huge difference in average claim size of network and non-network providers for treatment of similar disease, the distribution of claim coming from each becomes important and we need to take corrective actions to check any unfavourable development. Sometimes, agreement for fixed package rates with network hospitals are entered for 3-4 years and then rates are revised (usually upward). Therefore, there can be sudden spike in average claim size in one year and then it may remain moderate in future. Hence, before predicting average claim size for future we need to look into various aspects of how network management is done. It is also important to find out utilization of each kind of hospital, namely, primary, secondary, tertiary, super-specialist, etc.

[10]

Solution 5 :

- 1) The insured will have a choice between two ways of applying for the cover (insurer's name/product) provides.
 - 1A Full Medical underwriting
 - 1B Moratorium underwriting

(1A)**Full Medical underwriting**

- The prospect will need to complete a health questionnaire.
- The insurer will review the information provided and decide on the terms to be offered to the insured.
- If the prospect has a pre-existing condition that may need treatment in the future, the insurer will usually exclude it from the cover along with any conditions related to it or accept the application with a loading.
- The exclusion, if any, will be shown on the policy schedule the insured will receive from the insurer when the application has been processed. The same process will also apply for any members of the insured's family included in the application.
- If exclusion is applied to exclude treatment of a pre-existing condition at the time of policy inception, in some cases, the exclusion can be reviewed in future if such a request is received from the insured.
- If a loading is applied in view of a declared pre-existing condition, this loading will be for lifetime of the policy. No review is usually possible.

(1B)**Moratorium Underwriting**

- A customer choosing this option does not need to fill in a health questionnaire or health statement but will be automatically excluded for the cost of treating any pre-existing conditions during the first two years of the policy, for which they have received treatment and/or medication, or asked advice on, or had symptoms of (whether or not diagnosed), during the five years immediately before this insurance cover started.
- The conditions that could arise after the policy starts but are related to the pre-existing condition would also be excluded.
- If the customer has no symptoms, treatment, medication, or advice for any pre-existing conditions, and any directly related conditions, for 2 years after the policy starts, then such conditions will be covered after 2 years. However, if the customer has symptoms, treatment, medication, or advice for any pre-existing conditions, and any directly related conditions, within 2 years after the policy starts, then the treatment such condition and related conditions will be excluded for further 2 years i.e. for 4 years from the policy start date.
- The hospitalization cost of treating medical conditions that are likely to continue to need regular or periodic treatment, medication or medical advice, will never be covered by the policy.

(2)**Advantages of Moratorium**

- Speed and simplicity of application process is a major advantage.
- Works particularly well for people with a clean bill of health.
- If no health issues to PH, it saves time and administration.
- Only basic information needs to be provided by the prospect that is to be covered when they join and they will not be asked to disclose details of their medical history.
- If the customer satisfies the criteria of two years for a pre-existing condition, then treatment for that condition will automatically be covered should it later recur, subject to the policy terms and conditions, such conditions could include cataracts, joint replacements etc.

Disadvantages of Moratorium Underwriting

- Lack of certainty of what is, and what is not, covered especially when the insurer has got the right to rule on what illnesses are connected.
- Causes delay at the time of claim because of claims investigation.
- There is a danger that this type of underwriting may lead to individuals putting off medical intervention for two years so that a condition could be covered again. This could potentially damage their health.
- Significantly higher premiums as compared to the fully underwritten policy.

(3)**Challenges to the Insurer**

- It is open to misunderstanding and client gets the impression their pre-existing conditions will automatically be covered after 2 years.
- Poor knowledge of the agent/sales force for this type of product/uw approach.
- Poor medical records and also poor access to medical records.
- Mentality of Hospitals and doctors to help patient in all means possible.
- High degree of mis-selling to the prospect expected.

Possible solutions

- A careful and precise explanation of the rolling moratorium is essential.
- The sales force should be well trained and specialized channels to see this kind of product.
- The sales force should have much more involved conversations with customers and longer calls to sell the product.
- Stricter Policy Conditions
- Elaborate 2 year & 4 year waiting periods
- Permanent exclusion for chronic conditions which need regular or periodic treatment, medication or medical advice.
- Additionally, filter questions are increasingly used to ensure that only cleaner risk can be offered moratorium underwriting.

- Additionally, restricting availability to people with a BMI between 19 and 32.
- Restrict availability of cover through moratorium underwriting to younger lives – for e.g. up to the age of 40 years.

(4)**Possible reasons to try this approach**

- The disclosure rate in India is very low even under full medical underwriting approach.
- Most of the underwriting is still carried out at claims stage.
- With this approach, the dispute of non-disclosure is done away with.
- All pre-existing conditions to be covered after 4 years, as per the regulator's requirement.
- Only concern, is where declared, an application could have been accepted with an excluded, rated up or declined.
 - Excluded conditions would have to be covered after 4 years, as mentioned above.
 - Application which would have been rated up or declined will be accepted with no questions – so this can be priced for.

[21]**Solution 6 :****A-1) Renewable contracts**

- This means that the insurer is obliged to continue insuring the policyholder for the specified duration (term) of the policy, regardless of the policyholder's health status or claim experience during this period.
- The terms available on renewal may be identical to those applicable to a new entrant, with the same risk characteristics.
- Alternatively the renewal premiums may reflect the experience in the previous insured periods, via full experience rating for past experience.
- Renewable contracts are not guaranteed to renew after the specified term of the contract.

A-2) Reviewable contracts

- Reviewability refers to the degree to which premiums, that are guaranteed renewable under a long – term health insurance contract, can be changed in amount at the discretion of the insurer.
- Companies that undertake the reviews can be done annually, once in 3 year or 5 year etc.

- Reviewability relates to changes in the claim experience of the whole portfolio. It does not allow premiums to be raised for a particular policy that have a bad claims history while leaving the other policyholders with unchanged or lower premiums.
- Reviews of premiums can result in “selective lapsing”.

B) General Insurance and standalone companies are treated at par –

Companies	Life Insurance Companies	General Insurance / Standalone Health Companies
Medical Reimbursement Benefits	3 year renewable contracts with guaranteed renewability	Annually renewable contracts
Fixed Benefits – Hospital Cash & Surgical Cash	3 year reviewable with guaranteed renewability	Annually renewable contracts

[6]

Solution 7 :

A) Exclusions

- Exclusions can be related to pre – existing conditions – these are those conditions that the applicant has suffered previously and will often include related conditions as well.
- Other common exclusions are –
 - Criminal acts
 - Drug abuse
 - Hazardous sports and pastimes
 - Self-inflicted injury
 - War & civil Commotion
 - HIV / Aids

B) Issues to be considered and discussed while discussing IRDA’s circular –

Underwriting Freedom

Onus on the insurer to set out its u/w policy and that it subsequently ensures adherence in the case of policyholders with HIV or AIDS (PLWHA) cases.

Does the draft seem to suggest that insurers can design widely different policies in respect of accommodating PLWHA exposure?

From a clinical commercial perspective, an actuary might want to design an overly tough approach, so what restrictions are placed by the draft on the same?

The additional loadings that will be applied (once approved) may be widely differing between insurers - potential anti-selection risks for those insurers with the weakest restrictions.

In seeking approval, will the IRDA expect the insurer to rely on statistical, actuarial or other reasonable data to support their claim?

If so, what data is likely to be available?

Lack of Data/ Pricing

Clearly one of the most critical concerns with this proposal is lack of data.

Great social consideration on the part of the regulator, but how insurers will be able to express their perception of risk in such a structured manner, with differentiated loadings for various risk segments and degrees of severity of HIV/AIDS.

Even if international studies are available, these are not likely to be comprehensive for this purpose and clearly are not India specific.

Pricing risk is central here, but what about other risks, e.g. reserving risk, effective capital management (even worse when India moves towards RBC model), anti-selection, moral hazard, outright fraud.

Equity/ Social Inclusion

Insurance company must use every opportunity to remind the IRDA that insurers are in the business of taking risk and in doing so aim to target a reasonable profit over a sustainable time horizon.

There are goods and services in any economy that are good/bad for consumers and profitable/unprofitable (or perhaps less profitable).

Segments like this could arguably be lumped into the "good/unprofitable" quadrant and this is where the government needs to take responsibility and grapple with the provision/price dilemma, not forced onto the private insurance market.

Ultimately, there will have to be cross-subsidization between low-risk and high-risk (PLWAH) groups.

Accessibility and affordability has been a pressing issue for India and one that the Central Government has arguably tackled effectively with schemes like RSBY.

These schemes have been successful in targeting segments of the population that require health protection and historically would not have been the target market of health insurers.

Large pools of risks on a State-wide basis have been covered in a way that allows insurers to benefit from large diversification and almost-mandatory style coverage.

These schemes neither have any underwriting restrictions in the form of waiting periods, nor do they have any exclusion including HIV/AIDS.

Low sum insured large diversification type schemes may be a better vehicle to service these risk needs.

What lessons can we learn from other South Asian markets and for that matter across the globe?

Underwriting ramifications

Sexually transmitted diseases (STDs) are a standard exclusion on all private health insurance (non-government) schemes, so with a large proportion of all HIV infections arising through sexual transmission, how does this leave the existing underwriting structure as far as STDs are concerned.

Do we move to a situation where STDs become STDs excluding cases where HIV occurs - not particularly intuitive? Will insurers find themselves falling foul of any inequality laws?

Claims stage underwriting will become very complicated and reputational risk considerable for insurers declining claims through material non-disclosure.

Insurance companies can almost picture the media coverage of dying patient left harassed by insurer at his one time of need.

Financial Burden/ profitability

Insurers will manage profitability with a shift in subsidization by "old risks" in favor of "PLWHA" risks.

With guaranteed renewability and limited capacity to increase rates unless at a portfolio level (and with good justification), the problem and risk becomes amplified.

Disclosure & Confidentiality

Our country is not known for its ability to showcase high disclosure rates.

Approximately the overall disclosure rate is less than 6% and even worst for alcohol consumption and tobacco consumption, we can have a guess on what we think disclosure rates will be for HIV - probably as close to zero?

Our country is a very conservative society on matters of sexual health and the thought of constructing and applying a super-science underwriting policy against a backdrop of extremely low disclosure will invariably mean an approach where the enhanced risk is simply built into base rates.

IRDA support

With RSBY schemes, there is an effective information data exchange bureau which allows experience to be shared across the industry.

What will the IRDA do in general to promote data gathering and exchange in the context of PLWAH?

Should the Ministry of Health step in to be more pro-active in setting up data collection on HIV / AIDS across India?

Should the IRDA implement using an initial pilot basis where initiatives are only restricted to pilot products?

Implementation

Any major policy initiative requires much more time for all stakeholders to fully consider and engineer solutions.

The Insurance Council should approach IRDA in influencing on key issues and requesting more time and a reflection on what is important and doable.

Product Design

The more effective solution to the problem is to stipulate that all insurers writing health business have to provide some form of product that targets their needs.

Look for existing products available for example - relatively capped fixed benefit lump sum when HIV becomes AIDS and that too only on the basis of policyholder adhering to well defined HIV treatment protocols.

These norms can then be standardized across the industry.

One must assume that portability guidelines will apply any new PLWAH cover - this requires even more confidence across insurers.

The reference to product structures that provide an annuity in replacement of a lump sum seems a bit over the top for a health insurance that does want to play in the annuity/mortality game. Under CI, would the trigger be HIV diagnosis or onset of AIDS? Possibly further permutations?

Clarifications would be required for whether all health insurance schemes captured by this ED (e.g. retail, corporate, government; medical indemnity/reimbursement, hospital and surgical cash, PA, etc.)

[23]

[TOTAL MARKS – 100]
